Michael Michael

1. 19yo student who is a mod myope ~5.75DS contact lens rx has trouble reading with CL. what is your main concern? what would you do to try and rectify the problem? why would someone like her prefer CL over glasses?

Julie's answer
19yo student she has trouble accommodating but at that age
theres a lot of studying involved so like patient care and addressing their needs then i said ideally would watn their spec rx and see if vertex distance was adjusted for theyre not overminused etc, then try push as much plus in the distance as i can whilst still maintaining decent vision
so it would act like a mini add
in a way iunno if thats right
for why would she prefer CL > glasses i said cosmetic, better FOV, not as much minification, less aberrations but he wanted something else idk

VK

- Myopes accommodate and converge more through CLs (compared to spectacles) - Ensure patient is not over minused, ensure vertex distance was adjusted correctly - Greater convergence needed, concern would be convergence insufficiency→ treat CI before giving the CLs, through VT (e.g. Brock string)
    - CLs don't have the BI effect, thus myopes converge more through CL than specs - Greater accommodation needed in CLs---> _____
    - VT??
    - Monovision?

2. RGP keratoconic fit - what type of keratoconus are there? how would you differentiate between them? in what circumstances would you consider spectacles, CL, RGP? why can you not get 6/6 vision in a moderate keratoconic through the phoropter

VK

- Puberty onset keratoconus
    - Onset in early adolescence
    - Bilateral and asymmetric
    - Progression for the next 8-10years

- Late-onset keratoconus
    - Onset late 20s or early 30s
    - Bilateral, little assymetry??
    - Less severe progression
- Forme Fruste keratoconus
    - Onset at any age
    - Mild form, stops at a certain point
        - No refractive changes for 3-5years
            - Annual corneal topography shows no change over 3 subsequent meausrements
    - If pt is aged ~40yrs and stable refraction, forme fruste more likely than progressive KC
- Management of keratoconus
    - Spectacles for mild or early KC
    - CL- hydrogel
    - RGPs- when BCVA with specs can be improved using RGP
- Moderate keratoconus= 48-54D
- Cannot get moderate KC 6/6 through a phoroptic

Julie's answer
or the keratoconic one basically refraction designed to correct at 90 degrees in terms of astigmatism but keratoconus not liek tha

3. showed me some soft CL fit with a blob moving up and down in the pre-lens tear film? EW lens during like the 2nd week of wear when its a monthly lens. what can this blob be? what can cause it? what other signs on the eye would you see

Julie's answers
blob wasn't really clear... but like im pre sure it was a lipid deposit, oily af tear film, cause was build up of makeup that wasn't washed away properly, evert lids an check for papillae

ACTUALLY i think this

🧠 Context

When a contact lens is on the eye — especially a **soft lens** — the **pre-lens tear film** (the thin tear layer *on top of the contact lens*) starts to **break up and evaporate**.
If you look at it under **white light with the slit lamp**, you may see **coloured bands or fringes** moving across the lens surface.

🌈 1. "Lipid-like interference fringes"

- Normally, **lipid layer patterns** (from the natural tear film) show beautiful coloured interference fringes — pink, green, blue — because the oily layer on top of the tears has variable thickness.
- However, with contact lenses, sometimes you see **similar-looking coloured fringes**, but these are **not actually from lipids**.
- They *look* lipid-like, but the cause is different.

💧 2. What's Actually Happening

These "lipid-like" fringes are caused by **interference** between light waves reflecting from:

- the **front surface of the contact lens**, and
- the **top of the thinning aqueous (watery) tear film** layer sitting on it.

When the pre-lens tear film **evaporates**, its **thickness becomes uneven**, and light waves of different wavelengths interfere — producing **coloured patterns** (similar to an oil slick, but optically from thin-film interference).

🧪 3. Why It's Not True Lipid

- The **true lipid layer** sits at the very surface of the natural tear film.

- On a contact lens, that lipid layer often **doesn't spread properly** or may even be **absent**.
- So the coloured fringes you see **aren't from oil** — they're from **aqueous tear film thinning** due to **evaporation**.

Hence the phrase:

> "Coloured fringes due to tear film evaporation, not lipid."

🔍 4. Clinical Meaning

Seeing these coloured fringes means the **pre-lens tear film is unstable or thinning quickly** — a sign of **dryness or poor wettability**.
You might note:

- short pre-lens **TBUT** (tear break-up time),
- decreased comfort or fluctuating vision.

It's often an early indicator that the patient's **lens material, environment, or blink pattern** isn't maintaining adequate surface wetting.

🗣️ In a viva you could say:

> "The 'lipid-like' interference fringes seen on a contact lens are not from the lipid layer but from optical interference caused by thinning of the pre-lens aqueous tear film.
> They appear coloured because evaporation makes the tear film thickness uneven, producing interference fringes similar to oil films.
> Clinically, this indicates tear-film instability and increased evaporation."

4. steep fitting lens - when you first blink it's clear then it starts to get blurry. Why?

like with flat lens, when you first blink its blurry then clear why

and i was like lid attachment cause edge lift then recentres so vision clear but with the steep one the pooling induces a plus powered lens so that makes the vision blurry but when you blink you squeeze some of the tears out so its clear
powered lacrimal lens*

then i got some MCQ
- which of the following isb est siuation for soft toric - WTR astigmatism without any sphere, ATR, irregular astigmatism, keratoconus

Joseph's Questions from Michael Michael

1. Patient Rx -3.50/-0.75x40 - they would like to be fitted with a soft toric lens. Explain the procedure you would fit this patient
2. What are 3 DD SiH currently on the market?
3. What are 3 SiH that are not DD. rank these in terms of Dk/t. and what approcimate Dk/T are each of these
4. Shows my static RGP fit. Describe the fluorescein pattern (was a steep fit). You want to change BC from 7.6 to 7.8 now, current lens is 7.6/9.6/-3DS. What is the new lens? 5. RGP calculation. Cornea is 43@170 and 45.5@80. Whats the corneal cyl. Patient's Rx is now -5.75Ds, what RGP would you fit and why. The optom has chosen a spherical RGp, what is the residual cyl?
6. Two MCQs, one about epithelium healing time, second i forgot

Judy
1. 39yo diabetic chef, first time CL wearer, uses anti allergy drops, gave K readings and spec rx of -6.00/-0.25x180 and -6.50/-1.00x20. Which lens do you choose and why and parameters etc

Lisa's answer
Diabetes means he has a thinner epithelium, slower wound healing so need more oxygen. Low water content because he's a chef so don't want it to dry out. Wait five mins before putting in CLs after allergy drops
Don't swim in them and if you need then use goggles
Hence go for silicone daily disposable OD -5.50 OS -6.00/-0.75X20 in MyDay (sphere and toric)
2. rigid lens, what are the parameters called BOZR:BOZD/ICR:ICD/PCR:LD. and if you change

the base curve from 8.2 to 8.3 and rx is -3.00D, what rx should you order Lisa's answer" -2.50D as making the BC flatter makes the post lens tear film more minus hence need to reduce rx and 0.1mm~0.50D

3. a picture of striae, asked signs symptoms causes how to manage and ddxes So first section is a scenario, second section is rigid stuff and troubleshooting and third section is therapeutic stuff but not drugs, just contact lens related mgmt

Questions from Judy

1. what lens would you use and why that brand and type of material,and justify why like monthly compared to daily etc
2. describe an rgp fit and also to change the paramaters
3. last question was just "what is this", CL complications, mine was CL flutin, loose lens

Questions from lily

1. Rgp power crosses
2. What lens could you use for patient with +1.00/-5.00X160 Add +3.25 (similar rx of low sphere high cyl high add) and what considerations must you take into account. a. Proclear toric XR multifocal only option in soft lenses. The low sphere with high cyl will mean the lens is likely to rotate a lot. The prism ballast and minus power being along the vertical mean the inferior area will be quite thick and hence lower localised dk/t.

VK's Q's from Michael kang

- Case was SEAL→ DDx, DX, Mx

Eric Papas
46YO patient comes in having trouble reading > CL options to manage
Patient is now toric MF > what lens
Patient experiencing discomfort > why
Why would you opt for SiH vs. hydrogel

Jason's Questions from Eric Papas
What are the ingredient in Puremoist? Other than the active disinfecting agents what other chemicals are in it?
Pros and Cons of MPS? Somewhat compare to Peroxide solutions and other options
Benefits of hydrogel of SiH. When would you use a hydrogel over SiH? Is hydrogel neccsarily better for CLPC compared to SiH?
When would you pick rigid over soft lens? In general when is a rigid indicated? If px had lenticular astig and spherical cornea what rigid would you use? Hence what else would you need to consider

Hung's questions from Narelle
vertex power correction on meridians
when to use RGP over hydrogel (kc, grafs, OK)
2 main mechanisms of dry eye
MGD complications and amangement
3 and 9 olcock staining
CL lens availabilities
how to get away with uncorrected cyl = ocular dominance
how to enhance a multifocal
claculating corneal toricity
how to assess the fit of a rgp: dynamic v.s. static fitting (steep v.s. flat, or tight v.s. loose)
MMHH
knowing when to use overspecs
knowing when to use Proclear toric MF
knowing when to use monovision
using rgp monovision
e.g. when you have presbyopic high cyl in oblique

Lily:

R: +6.50/-3.00 X 180
L: +6.00/-2.75 X 10

For this Px what would you fit them with? What are you thinking about when you're deciding what to fit them with?

Back vertex correct the script? Why does back vertex correction change the script? Why does it change the script in a certain direction?

(For soft lenses if you've selected specific soft lenses) What problems might you come across with this lens? (lots of what else questions) What other problems might you come across with this toric lens? (lots of what else questions).

(Once hypoxia is brought up), How does hypoxia affect the eye? (explain effects on all layers of the cornea and other parts)

(Once microcysts are brought up) What are microcysts? Why do they appear? What happens when the contact lens is taken off? Where is happening to the material when microcysts go

away?

What else could you fit this patient with other than the lenses you have selected? (RGPs). What type of RGP? Why would you select this type of RGP? What are potential problems with RGPs for this particular patient? How can you improve these things?
These are the Ks for the patient, show me what lens you will fit for this patient. (After adjustment with Mandell-Moore factors), why did you flatten the curves? What happens if it is fit to the same curves as the cornea? Why did you change the back vertex power?